



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نهات علي اسماعيل علي

العنوان : ~

الحي / المدينة : عين شمس

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 19-09-2026

رقم التليفون : 01270216154

رقم البطاقة : 25101232601243

العمر (تاريخ الميلاد) : 23-01-1951 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك الإخصائي الاجتماعي (.....) التوقيع: (.....) المقر بما فيه التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة. تعتمد صحة البيانات المدونة في هذا النموذج صحيحة. رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: نيهات علي اسماعيل علي
National ID: 25101232601243



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bone scintigraphy

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

Name : saadeya mahmoud abdel zaher

Branch : Portable

Ref.Physician Prof : et cap+e

Date : 08-03-2026 11:33

Medical No. : GRI-32266

Ins.Company :

**MULTISLICE POST CONTRAST CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS:**

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported case of left breast cancer.

Compared to the last study dated 4/10/2025 ,the current study revealed:

(I) Chest:

- Left breast skin thickening .
- Few enlarged left axillary lymph nodes , the largest 11 x 7 mm .
- Progression of bilateral small pulmonary nodules, scattered at both lung lobes , the largest seen at right lower lobe measures 11mm .
- Bilateral lower lobar basal thin atelectatic bands are noted.
- The rest of both lung fields are clear showing no parenchymal nodules, cavitory lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Diffuse increased left breast density ,for sonomammographic assessment.

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous reduced CT density , still noted right hepatic lobe segment VIII subcapsular focal

Name : saadeya mahmoud abdel zaher

Branch : Portable

Ref.Physician Prof : et cap+c

Date : 08-03-2026 11:33

Medical No. : GRI-32266

Ins.Company :

lesion showing peripheral arterial nodular enhancement with no washout and measuring 16x8 mm ,likely hemangioma .

- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen (still noted two tiny non enhancing lesions measures 5mm ,likely small cysts), pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Dorsolumbar spondylotic changes.
- L4 degenerative spondylolisthesis.

OPINION:

- Progressive course regarding the pulmonary nodules .
- Rather stationary rest of the study.

Dr.Reham Kfiedr, MD

Patient's Name: SADYAA MAHMOUD
Date: 2026-03-17
Referred by: Prof.Dr.
Hospital: Goustave Roussey International Egypt



REPORT

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Left breast metastatic cancer on herceptin & hormonal therapy, follow up study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry, distortion or suspicious microcalcifications.
- Evidence of benign macrocalcification.
- Ultrasound demonstrated no solid or cystic lesions.
- Evidence of axillary accessory breast.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- Diffuse breast edema pattern in the form of coarsened trabeculae and thickened skin.
- LIQ posterior third, focal asymmetry with surrounding distortion with no suspicious microcalcifications.
- Evidence of benign macrocalcification.
- Ultrasound demonstrated 7 o'clock, area of granulation tissue abutting the skin, with no vascularity within.
- Evidence of axillary accessory breast.
- Normal nipple/areola complex.
- Non-specific axillary nodes with cortical thickness not exceeding 2 mm. No pathological nodes.

Conclusion:-

Right breast (BIRADS 2): Benign macrocalcifications.

Left breast (BIRADS 6): complete radiological response with noted diffuse breast edema pattern.

Best Regards,
DR. Amal Hegazi, M.Sc.



DMS
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١٥٥





Dmg

تكاليل

→ قهوه استرازدل

١٠٣٤ ← ٣٣٤
٦/١١ ٩/٢٩

→ موه
ليكو

فندق
تحت الجبل

١٠٣٥ ← ٧٦٠٣٤
١١٨
٦/١١





A- Patient ID:

1- Name: ليلى بنت محمد

Sex:

MDT form

2- Medical number:

03179847

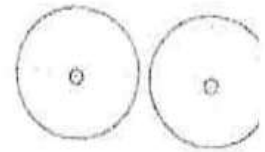
Old Med:

Old Med:

MSB1-2023

3- ID number: 25101232601243

Patient Age: 74 Y



Family History

Family History

Phone

Phone

01276886844

BMI

BP

REASON OF MDT

REASON OF MDT

Marital status

Marital status

HIS Score

HIS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co morbidity

Co morbidity

Menopausal status

Menopausal status

Initial STAGE

Initial STAGE

Date of MDT

Date of MDT

04/08/2

Breast Imaging:
If breast mass

Clinical data

Mets Workup:

ct cap: multiple mediastinal LN

bilateral pul. nodules

mild pericardial effusion

BM

ER

PR

Her 2 Neu

STSH

R167

IDC

8/8

6/8

3

40

RECOMMEND

RECOMMEND

RECOMMEND

RECOMMEND

RECOMMEND

RECOMMEND

T

N

M

LN

lung

Sonomammography

PATIENT HISTORY

ct cap: multiple mediastinal LN

bilateral pul. nodules

mild pericardial effusion

received 15 wks taxol + 6 cycles iv herceptin with RD in mid cycle evaluation

pt c/o neuropathy GIII not responding to treatment

MDT OUTCOME & PLAN

Continue on AI with HERceptin.

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

SIGNATURE

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition



Patient Name: Nbhat Ali Ismail Elewa	Date : 21/11/2024
Exam : C.T.CHEST	Age : 74 Year(s)
Medical ID: 4881/2023	Pat. ID: 45681

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN,
AND PELVIS: -**

compared to last study dated 20/7/2024

I Chest:-

- Rather stationary course of Prominent left axillary level I and II lymph nodes, the largest measures about 1.4 X 1 cm.
- Newly developed Right apical upper lobe and bilateral lower lobes scattered nodular and petechias of ground glass densities.
- Still noted Bilateral scattered pulmonary nodules, largest is at the lateral segment of middle lung lobe measuring 5 mm.
- No pathological mediastinal or hilar lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Still noted minimal pericardial effusion.
- Still noted left thyroid dense calcified nodule.

II Abdomen & pelvis:

- The liver is enlarged size with homogenous CT texture. No focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and still noted bulky both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus, otherwise normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Scanned skeleton:

Still noted sclerotic area seen at body of sternum.

Still noted Left iliac bone sclerotic area, likely bony island

Opinion:

- Newly developed pulmonary nodular and patches of ground glass densities, likely inflammatory.
- sclerotic area at sternum body for bone scan.
- stationary course of the rest of the study.

Prof. Menna Talla Hanafy, M.D.

Pathology Report

- Hospital number: #462-23
- Patient name: نيهات علي اسماعيل
- Nature of specimen: U/S guided tru cut biopsy
- Clinical diagnosis: Left Breast mass. (BIRAD 4c)
- Lab number: #462-23
- Referred by: --
- Report Date: 16-12-2023

Report

GROSS:

Multiple greyish tan tissue cores measured up to 1 cm long, totally submitted.

Microscopic :

Examination of serial sections revealed cores of breast tissue infiltrated by malignant ductal epithelial cells showing increased nucleocytoplasmic ratio, moderate pleomorphism and low mitotic index. Arranged in solid sheets and cords and poor attempt at tubular formation. Surrounded by moderate desmoplastic reaction and peritumoral lymphocytic infiltrate around 10%. Intraductal component is present about 7% in the form of solid and papillary pattern. Areas of hyalinosis are seen.

DIAGNOSIS:

Left breast mass, Tru cut biopsy:

Invasive ductal carcinoma Grade II,

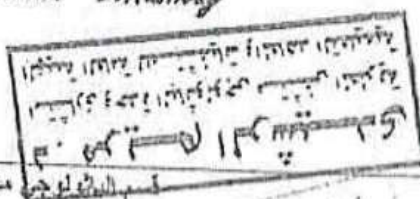
Invasive mammary carcinoma of no special type (NST), Grade II

Category core: B5

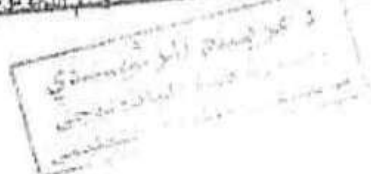
-Hormonal receptors in separate report.

SIGNATURE Dr. Marwan ElRashedy

Lab number: #462-23



مستشفى العظمية التعليمي





Pathology Report

- Hospital number: #462-23
- Patient name: نيفات عتي امساتيل
- Nature of specimen: U/S guided tru cut biopsy
- Clinical diagnosis: Left Breast mass. (BIRAD 4c)
- Lab number: #462-23
- Referred by:--
- Report Date: 16-11-2023

Report

Specimen:

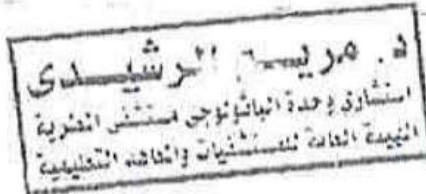
Internal Paraffin block labeled #462-23

Immunohistochemical results :

- ER: showed positive strong nuclear staining of 90% of tumor cells. (Allred score: 8/8) ✓
- PR: showed positive strong nuclear staining of 12% of tumor cells. (Allred score: 6/8)
- HER2NEU: showed positive strong membranous staining of >10% of tumor cells. (Score :3+) ✓
- KI67: showed positive staining of 70% of tumor cells. (High proliferation index)

SIGNATURE: Dr. Mariam ElRashedy

Lab number: #462-23





Patient Name: Nbhat Ali Ismail Elewa	Date : 17/04/2025
Exam : C.T CHEST	Age : 74 Year(s)
Medical ID: 4881/2023	Pat. ID: 45681

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN,
AND PELVIS: -**

- ↓ **Reported case of breast cancer.**
- ↓ **Compared to last study dated 21/11/2024**
- ↓ **Degraded quality of examination due to inevitable patient's motion artifact.**

I Chest:-

- Rather stationary course regarding size and number of previously noted prominent left axillary level I and II lymph nodes, the largest still measures about 1.3 X 0.9 cm.
- Rather stationary course regarding right apical and bilateral lower lobes scattered tiny nodular and patches of ground glass densities.
- Still noted bilateral scattered pulmonary nodules, largest is at the lateral segment of middle lung lobe measuring 5 mm.
- No pathological mediastinal or hilar enlarged lymph nodes.
- Still noted mild cardiomegaly.
- No evidence of pleural effusion, thickening or masses seen.
- Minimal regressive course of previously noted minimal pericardial effusion.
- Still noted left thyroid dense calcified nodule.

II Abdomen & pelvis:

- The liver is enlarged with homogenous CT texture. No focal lesions (on non-contrast basis) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and still noted bulky both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Left pelvic phleboli.
- Bulky uterus, otherwise normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Scanned skeleton:

- Newly developed right proximal femoral inter-trochanteric comminuted fracture noted.
- Dorso-lumbar spondylosis.
- Still noted sclerotic area seen at body of sternum for bone scan.



Patient Name: Nbhat Ali Ismail Elewa	Date : 17/04/2025
Exam : C.T CHEST	Age : 74 Year(s)
Medical ID: 4881/2023	Pat. ID: 45681

- Still noted Left iliac bone sclerotic area and smaller foci noted at right humeral head and left femoral neck, likely bony island.

Opinion:

- **Newly developed** right proximal femoral inter-trochanteric comminuted fracture for dedicated study and further assessment.
- Stationary course of the rest of the study.

Dr. Shaimaa Khaled, MSc.

Dr. Walid Mohamed, M.D.

U21
1 / 2
7517.

Name : Nbhat Ali Ismail Elewa

Branch : Portable

Ref.Physician Prof : \N

Date : 17-12-2025 10:34

Medical No. : GRI-45681

Ins.Company :

MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Compared to the previous CT scan performed on 17-4-2025, the current study revealed:

I Chest:-

- Stationary course regarding the previously noted bilateral scattered pulmonary nodules.
- Still noted left axillary lymph nodes
- No pathological mediastinal or hilar lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Still noted minimal pericardial effusion.
- Still noted left thyroid dense calcified nodule.

II Abdomen & pelvis:

- The liver is enlarged size with homogenous CT texture. No focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and still noted bulky both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus, otherwise normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

Scanned skeleton:

Still noted sclerotic area seen at body of sternum.

Still noted Left iliac bone sclerotic area, likely bony island

Opinion:

- stationary course of since the last study.

Dr. Asser Ahmed \N

Patient's Name:

Nabhat Ali Ismail Ali

Date:

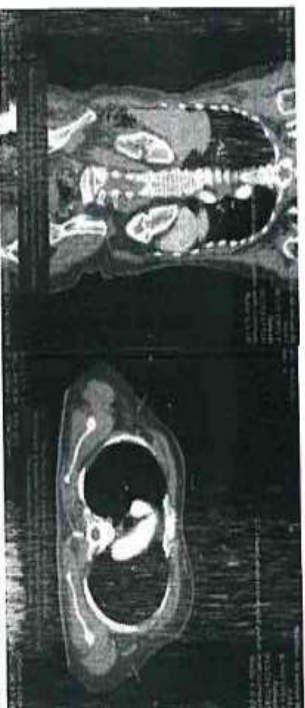
2026-08-10

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



**MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITH
CONTRAST ADMINISTRATION REVEALED:-**

- Persistence of the previously-seen left thyroid lobe calcified nodule.
- A newly-developed isthmus hypodense nodule is also seen and measures about 4.5mm.
- Bilateral axillary lymph nodes are still seen with loss of hilar fat contents in some of them on the left side and the largest one measures about 18 mm.
- Hyperinflated both lungs.
- Enhancing pretracheal lymph nodes are seen and the largest one measures about 15x9mm in dimensions.Calcification is also seen in one of them.
- Tiny calcified pulmonary nodules are still seen in the middle lobe and in the lateral segment of right lower pulmonary lobe . The former is larger and measures about 7.5mm.
- Thin linear atelectatic bands are seen in both lower pulmonary lobes which also shows variable sized bulla ,with the largest one seen in the left lower pulmonary lobe,posteriorly and measures about 22x15mm in dimensions.
- Bilateral increased broncho-vascular markings with basal bronchiectatic changes.
- No patchy areas of consolidations noted.
- No other sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural effusions.Yet ,bilateral mild pleural thickening and /or reaction is seen ,more on the left side.
- Mild pericardial effusion still seen.
- Average size of the heart with atherosclerotic changes of its big vessels.
- Persistence of the sclerotic foci in the body of the sternum,and left iliac bone with newly-developed similar lesions also seen in the anterior limb of the right acetabulum,left femoral head,and neck,and head of right humerus. Transfixation of the right femur is also seen.
- Average sized liver, measuring about 12.6cm ,showing a normal parenchymal attenuation and a homogenous enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.

Patient's Name: Nabhat Ali Ismail Ali
Date: 2026-08-10
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- Average sized spleen,measuring about 9.1 cm. No focal lesions seen inside.
- Relatively thickened both adrenal glands.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

IMPRESSIONS:

A status follow up ,and compared with the previous study done at 17/12/2025,the current study shows:

- Persistence of the previously-seen left thyroid lobe calcified nodule,denoting a stable course with a newly-developed hypodense nodule in the thyroid isthmus.
- Left axillary lymphadenopathy still seen with a stable course.
- Hyperinflated both lungs with enhancing pretracheal lymphadenopathy,
- Tiny calcified pulmonary nodules still seen in the middle lobe and in the lateral segment of right lower pulmonary lobe,denoting a stable course.
- Thin linear atelectatic bands in both lower pulmonary lobes which also shows variable sized bulla, bilateral increased broncho-vascular markings with basal bronchiectatic changes and bilateral mild pleural thickening and /or reaction ,more on the left side.
- Mild pericardial effusion still seen,denoting a stable course.
- Newly-developed relatively thickened both adrenal glands.

Prof//Hanan El-Alwal.

د. يوسف
روني
مستشار
رئيس
الطبيب

د. يوسف
روني
مستشار
رئيس
الطبيب



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

مبادرة
رئيس
الطبيب

09/13/2026

MDT FORM

Name:	NBHAT Ali Ismail ,	MRN:	575
Age:	76 Y	ID:	25101232601243
Menopausal Sta:	post	Phone:*	01276886844
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:		quilty dp for radiological review	
Surgical History:*	none	ECOG Performance Status	
		0 0 1 @ 2 0 3 0 4	

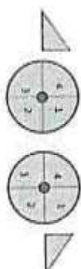
Clinical Data:

R

L

BX:

ER: PR: HER2: KI67:



T N M

History:

postm left breast cancer met to lung (initial cts pulmonary nodules largest 10mm)dc 2 er/8 pr/6/8 her23+ki 40% rec taxol herceptin st ai and continued herceptin till 8/2026

cts 3/2026 mild pd of pulmonary nodules largest 11 mm

cts 8/2026 ;pretaceal In 15x9 pulmonary nodules largest 7 mm

newly developed multiple sclerotic osseous foci
for mdt (moh no more approval of herceptin) and also for radiological
review of cts

Sonomammography:

Clinical Staging:

Biomarker/Molecular

Pathological Staging:

Mets Workup:

AFTER REVIWEING CTS >> STATIONARY DISEASE

Recommendations:

MDT Outcome & Plan

CONTINUE HERCEPTIN FROM NGO
AI FEMARA , ZOMETA /6 MONTHS
BASELINE BONE SCAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. يوسف روني	د. محمد توفيق		